

ALITO, J., dissenting

part.” §1395dd(e)(1)(A) (emphasis added).²

When a hospital determines that an “emergency medical condition” exists, it has two options. It may provide “treatment” within the capability of its “staff and facilities.” §1395dd(b)(1)(A). Or it may “transfer . . . the individual” to another hospital that “has available space and qualified personnel for the treatment” as long as the transfer would effect a net benefit for the patient. §§1395dd(b)(1)(B), (c)(2)(B)(i).

At no point in its elaboration of the screening, stabilization, and transfer requirements does EMTALA mention abortion. Just the opposite is true: EMTALA requires the hospital at every stage to protect an “unborn child” from harm.

Begin with the screening provision, which requires a hospital “to determine whether or not an emergency medical condition (within the meaning of subsection (e)(1)) exists.” §1395dd(a). “[W]ith respect to a pregnant woman,” subsection (e)(1) defines an emergency medical condition as one that is sufficiently serious to “plac[e] . . . the health of the woman *or her unborn child* . . . in serious jeopardy.” §1395dd(e)(1)(A)(i) (emphasis added). Thus, if the hospital identifies an emergency medical condition threatening the child, it must “stabilize” that condition to ensure that the child’s health does not remain in “jeopardy.” §§1395dd(b)(1)(A), (e)(1)(A)(i). It goes without saying that aborting an “unborn child” does not protect it from jeopardy.

Similarly, if a hospital wants to transfer a pregnant woman to another facility, it may not do so unless, among other things, a physician certifies directly or through an intermediary that the medical benefits of transfer outweigh

²At oral argument, the Solicitor General stated that, in the Government’s view, an “impairment” or “dysfunction” under §1395dd (e)(1)(A)(i) and (ii) may be temporary. Tr. of Oral Arg. 80.